

PE Rubric 4: The Pulmonary System Exam Rubric with Special Tests and OSE 26-27

Communication, Professionalism, and Patient Care
Introduced self, asked the patient name, preferred way to be addressed, DOB, and Age
Sanitized hands with air dry or washed hands at sink for 20 seconds, before AND after touching the patient and when appropriate
Obtained consent for the patient exam, acknowledged the patient's ability to relinquish consent at any time, and obtained permission to move draping to touch or examine skin (especially in sensitive areas) when needed throughout the exam
Supported the patient with comfort and empathy measures during exam: Checked to make sure that the exam was comfortable, elevated or lowered the exam table, offered assistance with changes in position, and used proper draping (if any of these things apply to the testing situation)
Spoke only to the patient during the exam, explained what was necessary, and used nonmedical jargon to explain things
Was respectful and professional in manner and demeanor throughout the exam to the patient and the faculty facilitator/proctor
Part 1- Pulmonary System Exams <i>(please tell the patient what you are doing and ask permission if removing garments in sensitive areas)</i>
UE Visualization and Palpation for Edema and Capillary Refill: Looks for signs of cyanosis, edema, capillary refill, clubbing, skin lesions, scars, rashes, bruises- verbalize to patient "I'm checking your skin and blood vessels" (needs to look at and palpate the entire arms, hands, & fingers) BL
LE Visualization and Palpation for Edema and Capillary Refill: Looks for signs of cyanosis, edema, capillary refill, clubbing, skin lesions, scars, rashes, bruises- verbalize to patient "I'm checking your skin and blood vessels" (needs to look at and palpate the entire legs, feet, and toes) BL
Appropriately Drapes the Chest: Obtains consent to move and open gown from the top or the side, to examine and listen to the chest, and it is done well
Visual Inspection of the Skin, Trachea, Chest, and Breathing: Must lower the gown to observe the anterior, lateral, and posterior rib cage during respiration; look for symmetry; Tell patient that you are looking at the chest/breathing to make sure that everything looks healthy/normal. Also look to see that the trachea is midline.
Auscultates Posterior Lungs (thorax): 7 points bilaterally posterior in an organized ladder, side to side and down, through each full breath with an open mouth (MUST include Bases and Lateral bases) 14 points in total on the skin (or under the gown)
Percusses Posterior Lungs (thorax): 7 points bilaterally posterior in an organized ladder, side to side and down (MUST include Bases and Lateral bases) 14 points in total on the skin (or under the gown)
Auscultates the Anterior Lungs (thorax): 6 points bilaterally anterior in an organized ladder, side to side and down, through each full breath with an open mouth (MUST include RML, Apical, and Bases) 12 points in total on the skin (or under the gown)
Percusses the Anterior Lungs (thorax): 6 points bilaterally anterior in an organized ladder, side to side and down (MUST include RML, Apical, and Bases) 12 points in total on the skin (or under the gown)
Part 2- Special Tests for Pulmonary: Choose at least 3 or More Best Special Tests to Perform based on Symptoms
Tactile Fremitus (for SOB or Cough): Palpation must include a minimum of 3 anterior BL and 4 posterior BL tactile fremitus areas in an organized ladder approach on the skin. Have the patient say "ninety-nine (99)" or "one-one-one (1-1-1)" with a normal voice volume . Tactile Fremitus is reported and documented as normal, increased, or decreased, and the region where this occurs is noted. A normal or neg- finding would be normal vibration to palpation; An abnormal or pos+ increased finding would be increased vibration to palpation, meaning that there might be a mass or consolidation present in that region; An abnormal or pos+ decreased finding would be less or no vibration to palpation, meaning that there might be fluid or an effusion present in that region.
Whispered Pectoriloquy (for SOB, Cough, Wheezes): Auscultation must include 6 points anterior BL and 7 points posterior BL in an organized ladder approach on the skin. Have the patient say "ninety-nine (99)" or "one-two-three (1-2-3)" with a whispered voice . Whispered Pectoriloquy is reported and documented as being not present or present, and the region where this occurs is noted. A not present or normal or neg- sound is heard as a muffled and indistinct whisper; A present or abnormal or pos+ sound is heard as louder and clearer, meaning that there might be a mass or consolidation present in that region; There also could be no sound at all, with effusion or fluid, and that would be abnormal or pos+.
Bronchophony (for SOB, Cough, or Wheezes): Auscultation must include 6 points anterior BL and 7 points posterior BL in an organized ladder approach on the skin. Have the patient say "ninety-nine (99)" with a normal voice . Bronchophony is reported and documented as being not present or present, and the region where this occurs is noted. A not present or normal or neg- sound is heard as a muffled and indistinct; A present or abnormal or pos+ sound is heard as louder and clearer, meaning that there might be a mass or consolidation present in that region; There also could be no sound at all, with an effusion or fluid, and that would be abnormal or pos+.

Egophony (for SOB, Cough, or Wheezes): Auscultation must include a minimum of 6 points anterior BL and 7 points posterior BL in an organized ladder approach on the skin. Have the patient say “EE or Key,” with a normal voice . Egophony is reported and documented as being not present or present, and the region where this occurs is noted. A not present or normal or neg- sounds like an “E or Key,” but a present or abnormal or pos+ sounds like an “Ay or Kay,” A present or abnormal or pos+ sound is heard when there might be a mass or consolidation present in that region; There could also be no sound at all, with effusion or no lung tissue, and that would be abnormal and pos+.
Diaphragmatic Excursion with Percussion on the Skin (for SOB, Cough, or Wheezes): Have the patient seated in an upright position take a deep breath and hold it. Percuss the back downward to where the percussion sound changes from hyper-resonant to dull. This is where the bottom of the diaphragmatic excursion is located during inhalation. Then ask the patient to exhale fully and hold it. Percuss up until the percussion sound becomes hyper-resonant again. This is the top of the diaphragmatic excursion during exhalation. Then measure the distance between the two lines. Normal diaphragmatic excursion should be about 3-5.5 cm. If the distance is less than this (3 cm or less), the diaphragm is not moving appropriately and this would be a positive test for a potential pneumonia or a tension pneumothorax. This test is done best BL
Respiratory Excursion (which in OMM we talk about as Bucket Handle Motion): Have the patient seated in an upright position and place your hands on the posterior thorax (chest) wall and with your hands and thumbs level at around T9 or T10. Observe the movement of your hands and thumbs while the patient takes deep breaths in and out. The thumbs should move 5-10 cm apart symmetrically with deep inspiration. An unequal/asymmetric chest wall expansion can be seen with atelectasis, pneumonia (consolidation), trauma, empyema, pleural effusion, or a pneumothorax, and this would be a positive test. This can also be performed in posterolateral and anterolateral positions as well.
Palpate for Head/Neck, Supraclavicular, Infraclavicular, & Axillary Lymphadenopathy (for SOB, F/C, Night Sweats, Wt. Loss, or Fatigue): Occipital, preauricular, postauricular, sublingual (tonsillar), submandibular, submental, anterior cervical chain, posterior cervical chain, deep cervical chain, supraclavicular, infraclavicular, and axillary lymph nodes BL (using good palpation technique)
Mouth Pain, Changes in Lymph Nodes, Swallowing, AND/OR Nicotine Use: Inspect AND Palpate Entire Oral Cavity (mouth) with a light, gloves on AND gauze
Check's for a DVT if SOB, Palpitations, Tachycardia, & Edema in one of the leg calves (to help find a cause for a Pulmonary Embolism or PE): Measure each calf at the widest spot with a tape measure for comparison/symmetry (if the size on one side is greater than 2 cm this might be significant); Homan's Sign for Hx purposes uses a forceful dorsiflexion of the foot with the leg extended causing calf pain is Not a very SN & SP, but still used by physicians even today.
Adam's Forward Bending Test (for Scoliosis): Have the patient slowly bend forward and observe for a rib hump or spinal curvature consistent with scoliosis. Then have the patient sidebend toward the deformity to see if it goes away. If functional scoliosis, the sidebending will reduce the hump.
Part 3- OSE TART and Boney Regional Testing: Choose at least 1 or More Best Exam Line to Perform for the Region based on Symptoms
Head & Cervical Spine Complaint: Screens for Symmetry, Palpates for TART, AND Segmental Diagnosis of OA, AA, AND Cervical Spine C2-C7 with N/Flex/Ext
Thoracic Spine Complaint: Screens for Symmetry, Palpates for TART, AND Segmental Diagnosis of the Thoracic Spine T1-T12 with N/Flex/Ext
Rib Complaint: Screens for Symmetry, Palpates for TART, 1 st Ribs with Thoracic Inlet AND Rib Motion inhaled & exhaled x 3 places AND anterior/posterior BL
Lumbar Spine Complaint: Screens for Symmetry, Palpates for TART, AND Segmental Diagnosis of the Lumbar Spine L1-L5 with N/Flex/Ext
Upper Extremity Complaint: Screens for Symmetry, Palpates for TART, AND Screens for Joint Motions Flex/Ext/AD/AB/IR/ER BL
Lower Extremity Complaint: Screens for Symmetry, Palpates for TART, AND Screens for Joint Motions Flex/Ext/AD/AB/IR/ER BL
Part 4- Example Associated System and Vascular Exam Testing: Choose Best Additional Exams to Perform for the Region based on Symptoms
Perform HEENT System Exams with Special Tests
Perform Cardiovascular System Exams with Special Tests
Perform MSK System Exams with Special Tests
Perform GI/GU System Exams with Special Tests
Perform Psych System Exams for Patients with SOB (Anxiety or Panic)